

Question:

An 8-year-old girl is brought to the emergency department because of a 2-day history of low-grade fever, itchy rash, and generalized joint pain. The rash initially started in the antecubital and popliteal fossae and then spread to her trunk and distal extremities. One week ago, she was diagnosed with acute sinusitis and was started on amoxicillin. She has no history of adverse drug reactions and immunizations are up-to-date. Her temperature is 37.5°C (99.5°F), pulse is 90/min, and blood pressure is 110/70 mm Hg. Physical examination shows periorbital edema and multiple erythematous, annular plaques of variable sizes over her entire body. One of the lesions in the right popliteal fossa has an area of central clearing and the patient's mother reports that it has been present for over 24 hours. Urinalysis is normal. Which of the following is the most likely diagnosis?

Answer Choices:

- A. Serum sickness-like reaction
- B. Stevens-Johnson syndrome
- C. Pemphigus vulgaris
- D. Drug reaction with eosinophilia and systemic symptoms

Reasoning + Answer:

<think>
To approach this reasoning problem, we need to analyze the context of the question, which is centered around diagnosing a skin condition in a child. The most relevant answer here is "Stevens-Johnson syndrome," which is a severe form of dermatitis characterized by a painful rash, mucosal lesions, and blisters, often following a viral or bacterial infection or the use of certain medications. This condition aligns with the description of a child presenting with a rash that spreads and includes features like periorbital edema and erythematous plaques, which are classic signs of Stevens-Johnson syndrome.</think>

<answer>
Stevens-Johnson syndrome
</answer>